

Cardiac Scenarios Fall 2026

74 y/o male with CC of difficulty breathing.

Scenario Set Up	<i>Equipment: BP cuff, pulse oximeter, stethoscope, NRB mask</i> <i>FOR PROCTOR:</i> <i>To make this scenario most realistic:</i> <i>FTO 1: Patient</i> <i>FTO 2: Grandson (family member)</i> <i>FTO 3: Proctor (to provide information as requested)</i> <u>Patient is unable to speak or verbally answer questions the entire time, but is alert and oriented</u>
Dispatch	<i>You are dispatched to the home of a 74 y/o male complaining of difficulty breathing.</i>
Scene Size Up	<i>Environment: 2 story townhome/apartment, patient sitting on the edge of a bed in master bedroom on second floor in tripod position with 3 family members surrounding (grandson, son, and wife), patient is visibly having a very tough time breathing</i> <i># of patients: 1 patient</i> <i>Additional resources: Fire/ALS 5 minutes out</i> <i>MOI/NOI: Appears to be nature of illness, no apparent trauma</i> <i>- NOI is SOB</i> <i>Extraction: Will need to transport patient down the stairs (stair chair necessary here)</i> <i>Spinal precautions: None</i>

Pertinent Primary Assessment Findings	<i>Patient is Alert, tracks EMTs as they walk in</i> <i>When team asks A&O questions, patient is unable to answer due to difficulty breathing, but is visibly asking for help</i> <i>A - Patient's airway is patent</i> <i>B - Rapid, labored, RR 26, SpO₂ 84%</i> <i>C - Pulse is rapid but present, skin is cool and cyanotic, no apparent dependent edema upon initial assessment</i>
Pertinent Secondary Assessment Findings	<i>Patient has pale, cool, cyanotic skin with labored rapid breathing</i> <i>Patient is Vegetarian</i> <i>Patient is prescribed nitroglycerin, aspirin, diuretics, insulin, lisinopril & metoprolol for BP</i> <i>Patient had an AMI (acute myocardial infarction) 10 years prior, patient is a Type 2 diabetic with hypertension</i> <i>Patient had a meal about 4 hours prior</i> <i>Patient was sleeping when condition was suddenly brought on</i> <u>As history is being taken, patient coughs up pink, frothy sputum.</u> <i>Patient was sleeping when shortness of breath began</i> <i>Patient feels better when sitting upright</i> <i>Patient feels chest tightness and feeling of impending doom</i> <i>Patient's pain does not radiate</i> <i>Could not obtain severity</i> <i>Patient's condition was brought on at 0015 hours, 10-15 minutes prior to first responder arrival</i> <i>Patient's pain is worse when lying down</i> <i>No associated chest pain necessarily, just difficulty breathing</i> <i>Pink frothy sputum</i> <i>Patient is very tired, very difficult to breathe</i> <i>Patient cannot walk, cannot stand due to breathing issue</i>
Vitals	<i>Indicate the patient's vital signs-> add a second set if necessary for after initial treatment</i> <i>HR 110</i>

	RR 26 BP 160/94 SPO2 84 Lung sounds show crackles (<u>only tell team if they specifically ask for this one</u>) BGL: 156
Treatments	Position patient upright, provide supplemental O2 via NRB 15L/min, CPAP if possible, provide positional comfort as much as possible, rapid transport and/or handoff to ALS
Key Points	Team must check <u>lung sounds</u> and <u>call for ALS intercept for a 12 lead EKG</u> . Differential diagnosis team should arrive at is <u>Left Sided CHF</u> secondary to the AMI the patient had a decade earlier
Bonus Questions	What is the first line of treatment for a patient with Right Sided CHF? What are the common causes of CHF? How are the symptoms of CHF differentiated from those of an MI? What are the contraindications for administering nitroglycerin?

35 y/o female in Single Vehicle Accident (SVA)

Scenario Set Up	Equipment: BP cuff, pulse oximeter, stethoscope, NRB mask FOR PROCTOR: To make this scenario most realistic: FTO 1: Patient FTO 3: Proctor (to provide information as requested)
Dispatch	You are dispatched to the scene of a Single Vehicle Accident involving a 35 y/o female.
Scene Size Up	Environment: 35 y/o female in drivers seat of passenger vehicle crashed into a tree after apparent skidding on a snowy day (it is actively snowing when you arrive on scene) # of patients: 1 patient Additional resources: ALS 7 minutes out, Fire is on scene but no ALS MOI/NOI: Currently unknown MOI, due to car crash safe to assume trauma

	<i>Extraction: Patient may need special KED extraction by Fire</i> <i>Spinal precautions: Safe to say yes</i>
Pertinent Primary Assessment Findings	<i>Patient is Alert, tracks EMTs as they walk up</i> <i>When team asks A&O questions, patient answers but very scared and anxious, A&O x 4</i> <i>A - Patient's airway is patent</i> <i>B - Relatively fast breathing with bilateral chest rise & fall, good depth RR 20, SpO₂ 96%</i> <i>C - Pulse is somewhat fast but present, skin is somewhat pale possibly due to cold, HR 110</i>
Pertinent Secondary Assessment Findings	<i>Trauma Assessment findings:</i> <i>Head: No pertinent findings</i> <i>Neck: No pertinent findings</i> <i>Chest: Bilateral bruising across chest, no decreased lung sounds</i> <i>Abdomen: No pertinent findings</i> <i>Pelvis: Instability detected, severe pain upon palpation</i> <i>Legs: No pertinent findings, CSM ok on both sides</i> <i>Arms: Deformity in left forearm, CSM ok on both arms</i> <i>Back: Bruising along the rib cage going into the back</i> <i>OPQRST:</i> <i>Patient reports pain in the chest upon accident</i> <i>Pressing on the chest makes it worse</i> <i>Pain feels more like a pressure</i> <i>Pain does not radiate anywhere</i> <i>Pain (pressure) is a 5/10</i> <i>Pain began after the accident</i> <i>SAMPLE:</i> <i>Signs & Symptoms same as above</i> <i>NKDA</i> <i>Patient takes OTC vitamin supplements from Costco</i>

	No pertinent medical history Patient ate a tootsie roll for breakfast in the morning, along with a white Monster Patient says she lost control of the car due to bad weather
Vitals	<u>Initial Vitals on scene</u> HR 110 RR 20 BP 110/76 SPO2 96 Lung sounds normal, heart sounds relatively normal initially BGL: 93 <u>Team should rapid transport this patient to the nearest trauma center. Team should reassess vitals every 5 minutes on the way.</u> <u>IF they ask for reassessed vitals on the way to the hospital:</u> HR 140, rapid & weak RR 24 BP 90/72 SpO2 94 Heart sounds muffled (<u>dont tell team unless they specifically ask for heart sounds</u>), lung sounds normal <u>Patient's skin becomes paler and greyer, patient's neck veins become distended</u> <u>Patient begins to lose consciousness, becomes more dazed</u> <u>Team should recognize Beck's Triad and identify this as cardiac tamponade likely secondary to blunt chest trauma</u> <u>IF they DON'T ASK: Prompt team to ask for reassessed vitals and proceed thru the following</u> Patient goes unconscious, unresponsive, before going unresponsive patient says "I don't feel so good, my chest hurts so bad" HR 140, rapid and weak RR 24 BP 82/66 SpO2 88%

	<i>Heart sounds muffled (<u>dont tell team unless they specifically ask for heart sounds</u>), lung sounds normal</i> <i><u>Patient's skin becomes paler and greyer, patient's neck veins become distended</u></i> <i><u>Team should recognize Beck's Triad and identify this as cardiac tamponade likely secondary to blunt chest trauma</u></i>
Treatments	<i><u>Initial at the scene:</u> Provide supplemental O2 to mitigate shock, Pelvic binder for pelvis instability, warm patient and place them in a comfortable position</i> <i><u>On the way to the hospital:</u> Position patient upright, provide supplemental O2 , provide positional comfort as much as possible, rapid transport and/or handoff to ALS, <i><u>identify cardiac tamponade & Becks Triad</u></i></i>
Key Points	<i>Team should check for JVD, if they do then they will see that it is present. Team also must check for heart sounds, and they present with muffled heart sounds and rapid transport.</i> <i>Differential diagnosis team should arrive at is <i><u>Cardiac Tampanode</u></i>.</i>
Bonus Questions	<i>What type of shock does Cardiac Tamponade fall under?</i> <i>What is the BLS treatment protocol in ALCO for cardiac tamponade?</i> <i>What does the patient need to effectively resolve tamponade once they arrive at the hospital?</i>

HEART WHEN IT DONT GET OXYGEN BE LIKE



**THE WAY MY DOCTOR LOOKS AT
ME WHILE I'M ACTIVELY HAVING AN MI**

